

POST TRAUMATIC STRESS DISORDER

- PTSD Is defined as Individuals who have been exposed to traumatic event in which person experienced, witnessed a death, serious injury or threat to physical integrity of self.

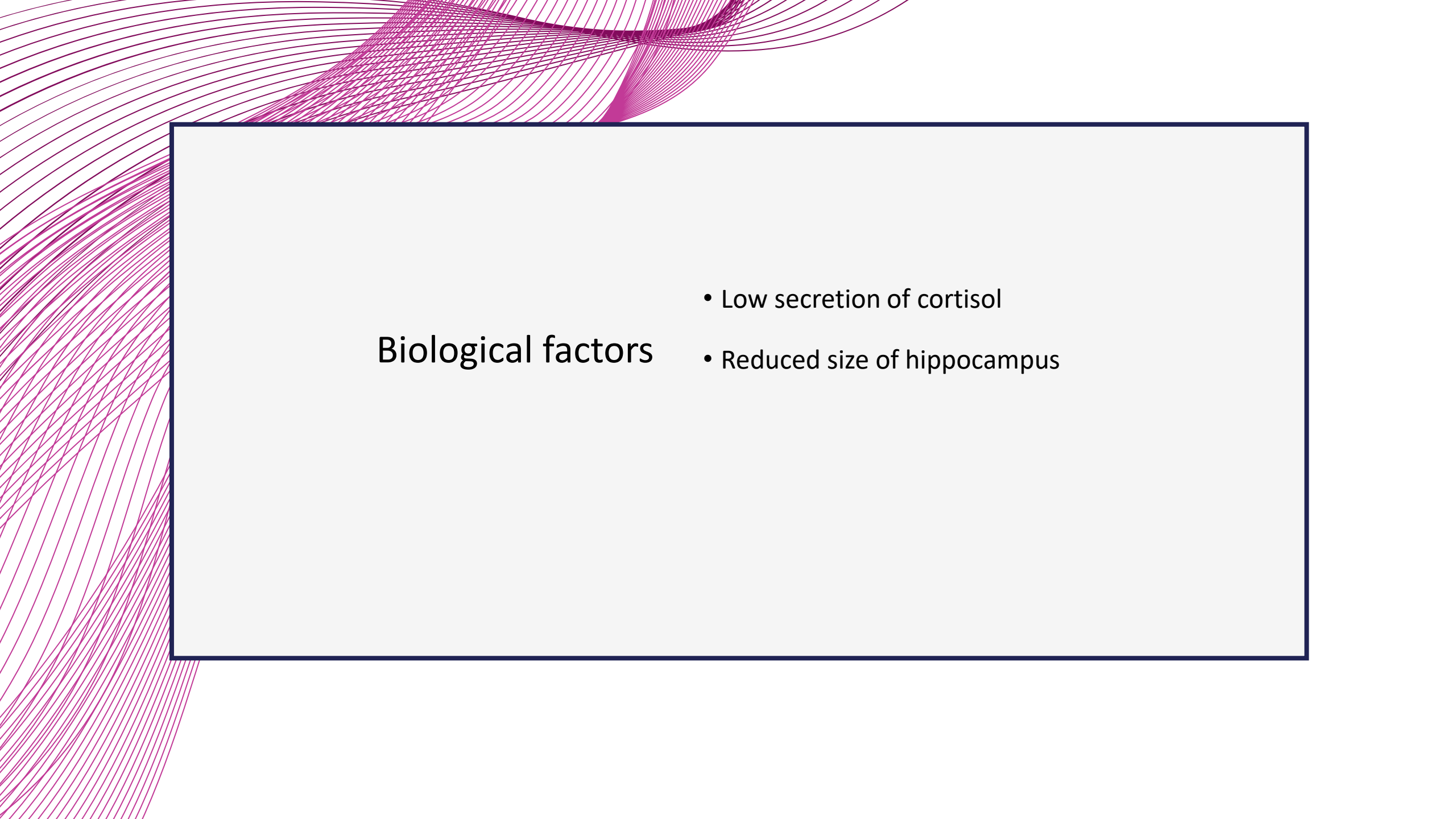


CHARACTERISTICS

- Experience of a traumatic event leading to the development of the condition
 - The traumatic event may be life-threatening, such as combat, a natural disaster a car accident, or sexual assault
 - But sometimes the event is not necessarily a dangerous one. For example, the sudden, unexpected death of a loved one can also cause PTSD.
-

Psychological factors

- Traumatic events that can lead to PTSD include:
- War
- Natural disasters
- Car or any other crashes
- Terrorist attacks
- Childhood neglect
- Kidnapping
- Rape
- Physical abuse
- Sexual abuse



Biological factors

- Low secretion of cortisol
- Reduced size of hippocampus

Signs and symptoms

- Intrusive or upsetting memories
- Nightmares
- Feeling of intense distress when reminded of the trauma
- Intense physical reactions to reminders of the event (increased heart rate, rapid breathing, nausea, sweating)
- Avoiding activities, places thoughts or peoples that remind the trauma
- Loss of interest in activities life in general
- Feeling of detachment from others, family and friends
- Difficulty in falling asleep
- Irritability
- Difficulty in concentration
- Hopelessness



Diagnostic Guidelines Acc. to DSM-5

A - Exposure to a traumatic event in 1 of the following ways:

- Directly experiencing the traumatic event(s)
- Witnessing, in person, the event(s) as it occurred to others.
- Traumatic event(s) occurred to a close family member or close friend.
- Experiencing repeated exposure to aversive details of the traumatic event



Diagnostic Guidelines Acc. to DSM-5

B - Presence of one (or more) of the following intrusion symptoms associated with the traumatic event:

- Distressing memories of the traumatic event
- Dreams related to traumatic event
- Occurrences of Flashbacks
- Distress due to resemblances of traumatic events
- Physiological reactions to resemblances of traumatic events



Nightmares.



Flashbacks.

Diagnostic Guidelines Acc. to DSM-5

C - Avoidance of 1 of the following:



- Distressing memories
- External reminders like places

Diagnostic Guidelines Acc. to DSM-5

D - Negative changes in thoughts and mood as evidenced by 2 of the following:

- Forgetting important aspects of the traumatic event
- Distorted thoughts about consequence
- Lack of interest in activities
- Detachment from others
- Inability to experience positive emotions



Forgetting important aspects of the traumatic event.



Unable to experience positive emotions.



Negative thoughts about yourself or others.

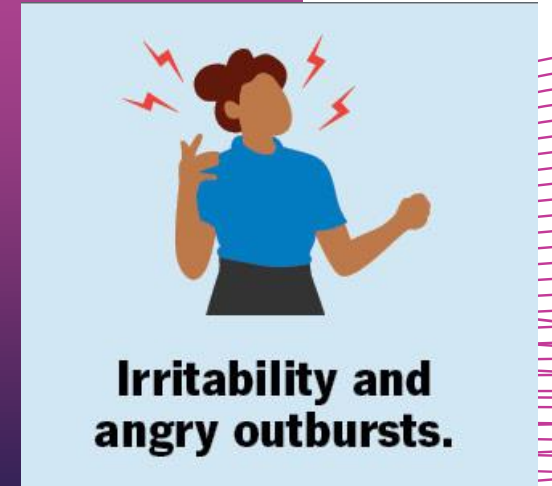
Diagnostic Guidelines Acc. to DSM-5

E - Marked changes in arousal and reactivity associated with the traumatic event as evidenced by 2 of the following:

- Irritable behaviour and anger outbursts
- Self destructive behavior
- Problems with concentration
- Sleep disturbance



Trouble
falling asleep



**Irritability and
angry outbursts.**

Diagnostic Guidelines Acc. to DSM-5

F - Duration of disturbance more than 1 month

G - Clinically significant impairment in functioning

H - Not due to effects of substance or another medical condition

Pharmacological treatment

01

Antidepressants

02

Anti anxiety

Psychosocial treatment

01

Trauma focused CBT

- It involves carefully and gradually exposing client to thoughts, feelings and situations that remind the trauma
- Identifying upsetting thoughts about traumatic event and helping to address those thoughts

02

Exposure therapy

- It involves gradually facing the thoughts and memories of the traumatic event or situations that one anxious
- This can be done using imaging techniques or by actually returning to the place where one had an accident
- Exposure should be done gradual and done with the help of an experienced clinician



Psychosocial treatment

03

Cognitive restructuring therapy

- Cognitive restructuring aims at replacing dysfunctional thoughts with more realistic and helpful ones
- Eg: I will never be normal again
- Replace: I will get better, it will just take time

02

Self help treatment for PTSD

- Reachout to others for support
- Avoid alcohol and drugs
- Challenge your sense of helplessness

References

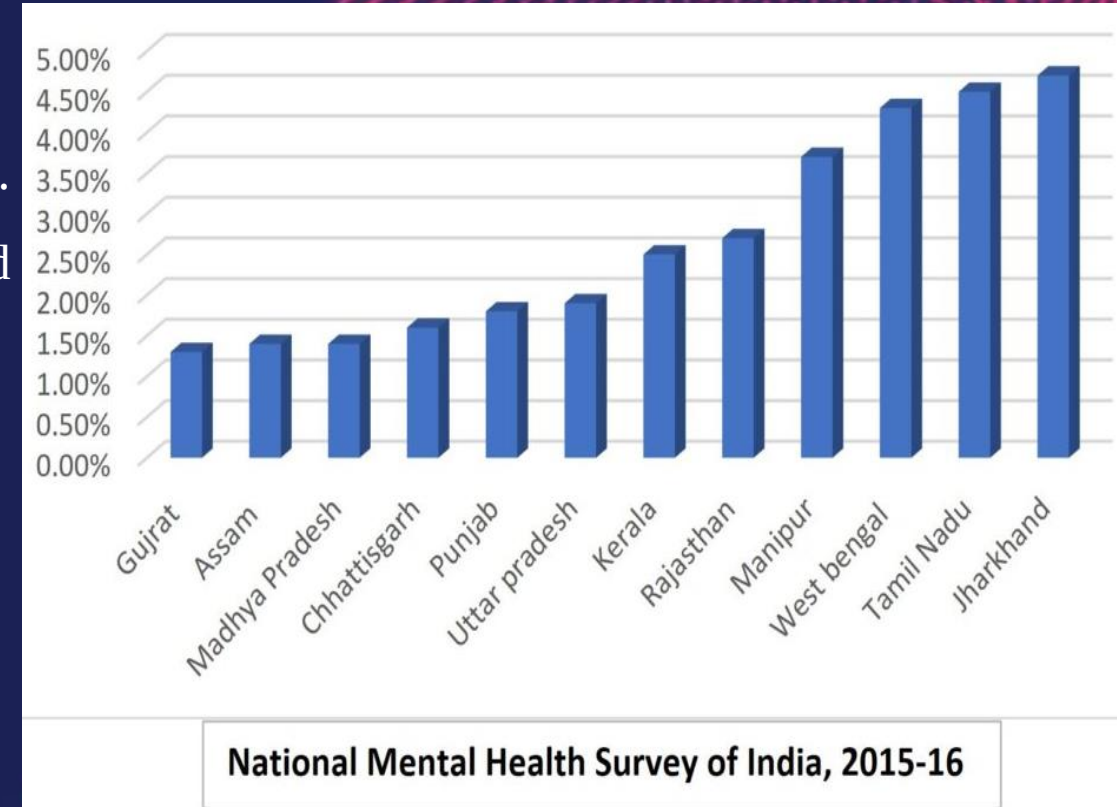
American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). <https://doi.org/10.1176/appi.books.9780890425787>

DEPRESSION

- Depression is also called clinical depression or major depressive disorder.
 - Its much more than simple unhappiness
 - Depression is a common and long lasting mood disorder that affects how you think, feel or behave and lead to variety of emotional and physiological difficulties
 - When people are in depression, they experience intense sadness, including feelings of worthlessness, hopelessness and helplessness which lasts for weeks or even months and interferes with your everyday functioning
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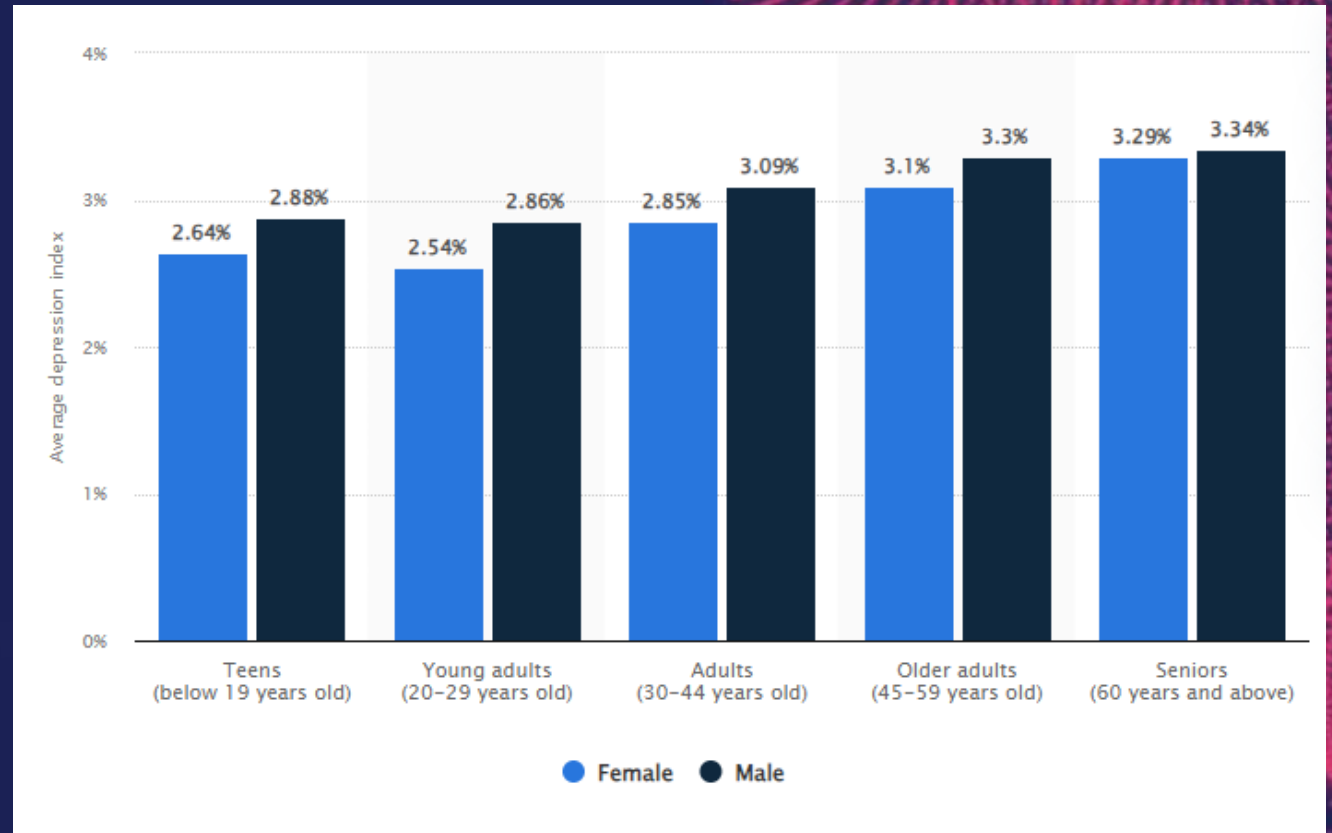
PREVALENCE

- Worldwide prevalence as of March, 2023 –
 - Approximately 280 million people (3.8%) in the world have depression (WHO, 2023).
 - Including 5% of adults (4% among men and 6% among women) (WHO, 2023).
 - 5.7% of adults older than 60 years (WHO, 2023).
- Nationwide prevalence - According to the 2015 NHMS survey, one in every 20 Indians was found to have suffered from depressive disorders at some point in their lives (NIMHANS, 2015)



PREVALENCE - GENDER

- Worldwide prevalence - Depression is about 50% more common among women than among men (WHO, 2023).
- Nationwide prevalence based on age and gender (Manya, 2023)



PREVALENCE – SOCIODEMOGRAPHICS (ARVIND ET AL., 2019)

Place of residence –

- Rural – 2.15% had depressive disorders
- Cities with population <1 million – 1.90% had depressive disorders
- Cities with population >1 million – 5.17% had depressive disorders

Occupation –

- Working – 2.70% had depressive disorders
- Not working – 2.66% had depressive disorders
- Marital status –
 - Never married – 1.70% had depressive disorders
 - Married – 2.75% had depressive disorders
 - Widowed/divorced/separated – 5.23% had depressive disorders



SIGNS AND SYMPTOMS

How you might behave

- Not able to concentrate
- Withdrawing from close family and friends
- No longer finding enjoyment in things that used to bring pleasure
- Not getting things done at school/work
- Relying on alcohol or sedatives

How you might think

- “I’m a failure”
- “No one likes me”
- “Things will never get better”
- “I’m worthless”
- “I can’t be bothered”
- “Others would be better off without me”

SIGNS AND SYMPTOMS

How you might feel

- Low/sad/unhappy
- Overwhelmed
- Low patience/irritable
- Upset
- Tearful
- Hopeless

How your body might respond

- Tiredness/lack of energy
- Poor memory
- Decreased pain tolerance
- Sleep problems
- Changes in appetite or weight



COMMON MYTHS ABOUT DEPRESSION

MYTH 1: You can simply 'snap out of it'

- Reality: No one chooses to be depressed. Someone who is depressed can't just shut it off.
- Depression is a psychological, social, and biological condition.
- If you suspect you are experiencing depression, contact your therapist or make an appointment with an experienced practitioner.

COMMON MYTHS ABOUT DEPRESSION

MYTH 2: Talking about it only makes it worse



- Reality: Being alone with your thoughts can be much more harmful than letting them out.
- Talking to a supportive, empathic, and non-judgmental listener has been shown to help a lot of people.
- An accredited therapist is best equipped to provide constructive support.

COMMON MYTHS ABOUT DEPRESSION

MYTH 3: Others are better at dealing with their lives, I'm just weak

- Reality: It may be because when you are feeling low, you tend to compare yourself with people who appear to be doing better.
- In reality, depression is not a sign of weakness or laziness. It has social, psychological and biological origins and can be treated in a variety of ways.
- Believing that you cannot cope is a common symptom of depression and does not mean you are weak.

COMMON MYTHS ABOUT DEPRESSION

MYTH 4: Depression is always triggered by something bad happening

- Reality: Sometimes it is difficult to understand how depression has developed. A lot of different factors can increase the likelihood of someone developing depression, including traumatic events such as big life changes, loss, and accidents.
- However, although traumatic events can be a potential trigger for depression, they are not the root cause of it. Depression may arise suddenly, even when things seem to be going well.
- In almost every case, an outsider like a therapist can help people identify possible factors contributing to depression and help them to stop blaming themselves for feeling that way.



COMMON MYTHS ABOUT DEPRESSION

MYTH 5: Depression is biological, there is nothing you can do about it

- Reality: Biology does play a role in the development of depression, but usually alongside a combination of other psychological and environmental factors.
- Even if it is partly biological, depression is treatable and there are many things that can be done about it.
- By making changes in the way you think and behave, you may be able to disrupt the vicious cycle of depression.
- Additionally, talking therapy does help many people and combining medication with talking therapy is a common treatment strategy for depression.



CAUSES

LIFE EVENTS:

- Early childhood trauma and losses
- Stressful adult life events (such as divorce, loss of a job, death of a loved one, family conflict, retirement)
- Experiencing several prolonged and severe difficult life events increases the likelihood for a person to develop a depressive disorder.

CAUSES

THINKING STYLES:

- Cognitive theory suggests that the way we think and how we interpret events and situations impacts how we feel and can lead to depression
- Overstressing the negative
- Thinking that you know what others are thinking and that they are thinking badly of you. we commonly think about things in a negative manner and this has a negative impact on our mood.



CAUSES

LIFESTYLE FACTORS:

- Not engaging in enough physical exercise
- Being over or underweight
- Having fewer social relationships
- Prolonged exposure to ordinary stress such as ongoing problems at work, difficulties in a relationship, or loneliness.



CAUSES



BIOLOGICAL FACTORS:

- Family history of depression are more vulnerable developing depression at some stage in their lives.
- Neurotransmitter imbalance

Situation

Samantha makes a mistake at work that costs her company a client

Thoughts

- I'm a failure
- I'm not good at anything
- Nothing will ever get better

Feelings

- Sad
- Upset
- Hopeless

Behaviour

- Stays in bed
- Can't concentrate
- Withdrawing from friends

Physiology

- Tiredness and lack of energy
- Sleep problems
- Poor appetite

WHAT CAUSES DEPRESSION GOING?

- When someone is depressed, they experience many changes. Often, these changes develop into a vicious cycle which keeps depression going.
- The vicious cycle of depression consists of a combination of thoughts, feelings, behaviours, and bodily sensations.
- a specific event triggered the cycle of negative thoughts.
- People who are depressed have a tendency to interpret events in a negative fashion.
- The negative interpretation of events is one of the important factors of keeping depression going.

WHAT CAUSES DEPRESSION GOING?

- In the above example, we can see how the clients thoughts about being a failure and not good at anything led to a change in her behaviour.
- She started staying in bed most of the time, had difficulty concentrating and began to withdraw from her friends.
- This further fueled her cycle of depression as it led to her feeling sad, upset, and hopeless about the situation.
- Her body also reacted to all these changes and she experienced tiredness and lack of energy, started having sleep problems and developed a poor appetite.
- Looking at this example more closely, we can see how Samantha's thoughts, behaviours, feelings and bodily sensations all interacted and combined to keep her depression going

Categories of Depression based on severity

01

Mild depression

02

Moderate depression

03

Severe depression with or without psychotic symptoms

Diagnostic Guidelines Acc. to ICD-10

Mild Depressive episode

- At least two of:
 - a) Depressed mood
 - b) loss of interest and enjoyment
 - c) Increased fatiguability
- At least two of:
 - (a) reduced concentration and attention;
 - (b) reduced self-esteem and self-confidence;
 - (c) ideas of guilt and unworthiness;
 - (d) bleak and pessimistic views of the future;
 - (e) ideas or acts of self-harm or suicide;
 - (f) disturbed sleep
 - (g) diminished appetite.
- Minimum duration of 2 weeks
- Some difficulty in continuing with ordinary work and social activities, but will probably not cease to function completely.



Diagnostic Guidelines Acc. to ICD-10

Moderate Depressive episode

- At least two of:
 - a) Depressed mood
 - b) loss of interest and enjoyment
 - c) Increased fatiguability
- At least three (preferably four) of:
 - (a) reduced concentration and attention;
 - (b) reduced self-esteem and self-confidence;
 - (c) ideas of guilt and unworthiness;
 - (d) bleak and pessimistic views of the future;
 - (e) ideas or acts of self-harm or suicide;
 - (f) disturbed sleep
 - (g) diminished appetite.
- Minimum duration of 2 weeks
- Considerable difficulty in continuing with social, work or domestic activities.



Diagnostic Guidelines Acc. to ICD-10

Severe Depressive episode

- All of these three:
 - a) Depressed mood
 - b) loss of interest and enjoyment
 - c) Increased fatiguability
- Atleast four of these with severe intensity:
 - (a)reduced concentration and attention;
 - (b)reduced self-esteem and self-confidence;
 - (c)ideas of guilt and unworthiness;
 - (d)bleak and pessimistic views of the future;
 - (e)ideas or acts of self-harm or suicide;
 - (f)disturbed sleep
 - (g)diminished appetite.
- Minimum duration of 2 weeks, but if the symptoms are particularly severe and of very rapid onset, it may be justified to make this diagnosis after less than 2 weeks.
- Significant difficulty in continuing with social, work or domestic activities.



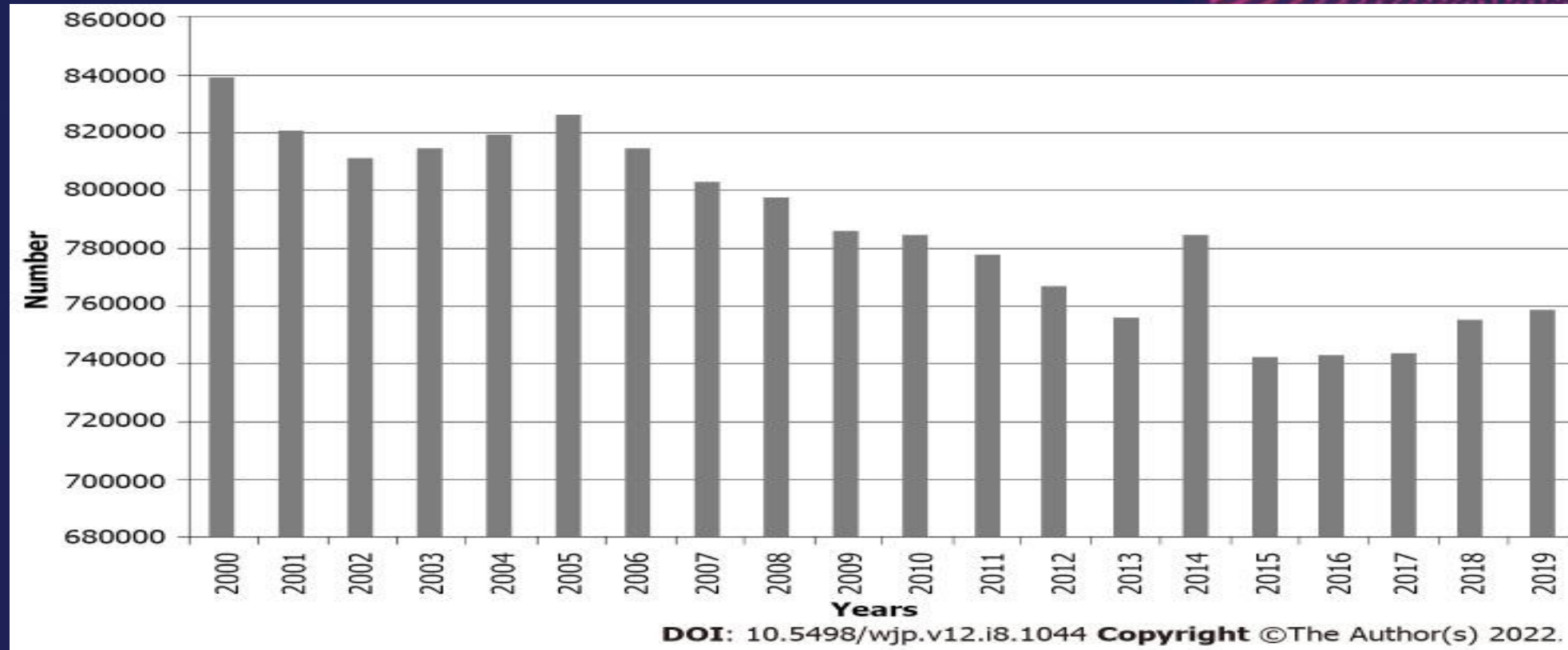
SUICIDE

(WHO, 2023)

- Every year 7,00,000 people take their own life and there are many more people who attempt suicide.
- Every suicide is a tragedy that affects families, communities and entire countries and has long-lasting effects on the people left behind.
- Suicide occurs throughout the lifespan and was the fourth leading cause of death among 15–29-year-olds globally in 2019.
- Seventy-seven per cent of global suicides occur in low- and middle-income countries.

PREVALENCE - WORLDWIDE

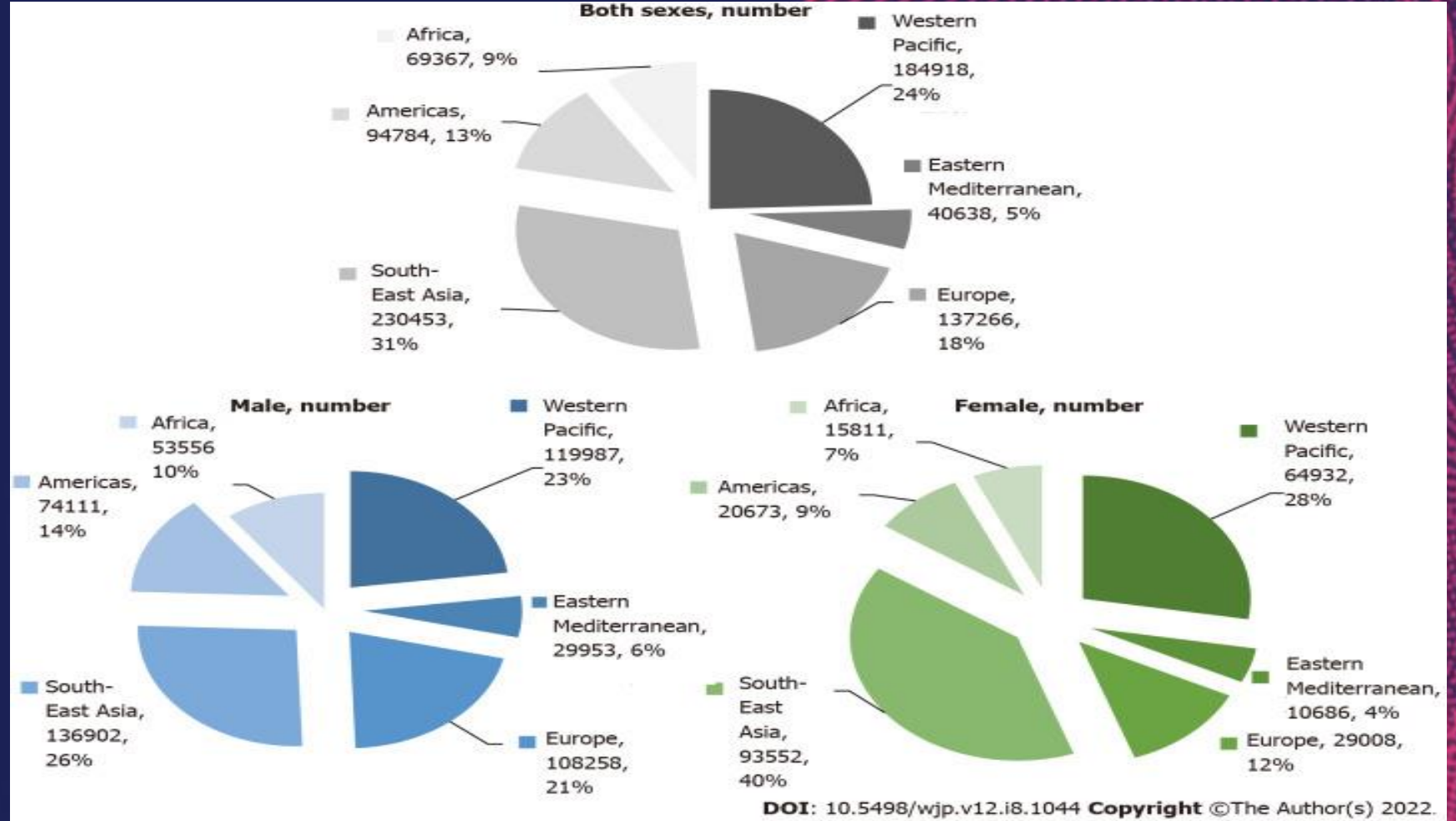
- Global suicide deaths, 2000-2019.



Source: World Health Organization and Global Burden of Disease estimates

PREVALENCE – GENDER (2019)

The data points out that around 26% of male in southeast Asian countries had committed suicides and for female this number is around 40%



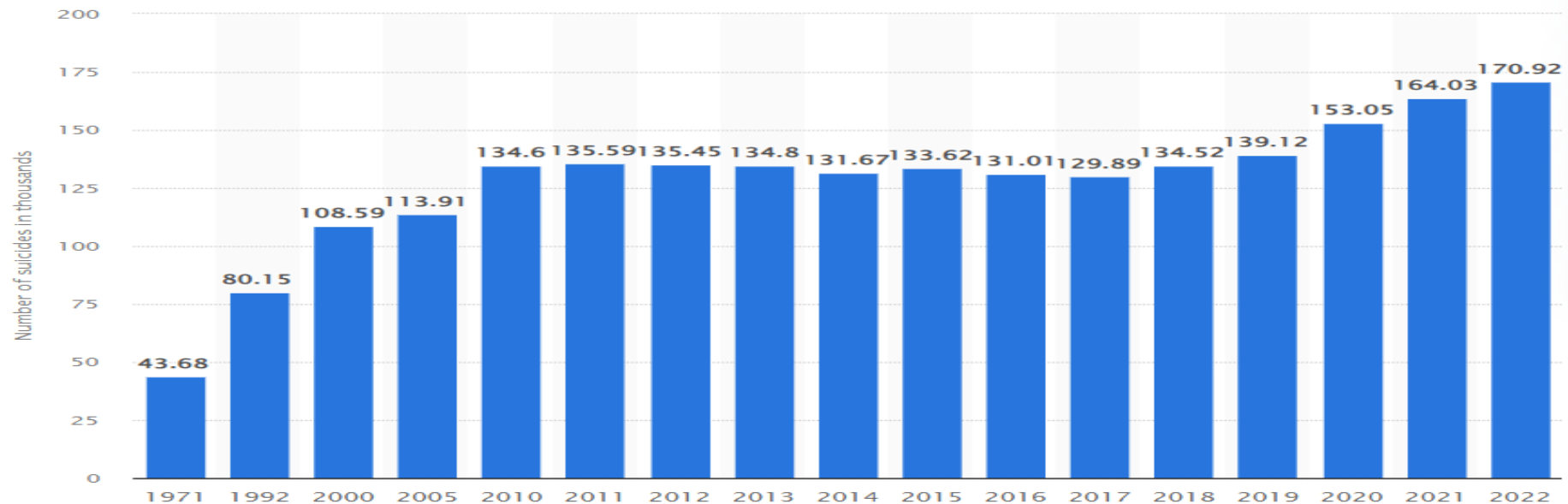
Source: World Health Organization and Global Burden of Disease estimates.

PREVALENCE – NATIONWIDE

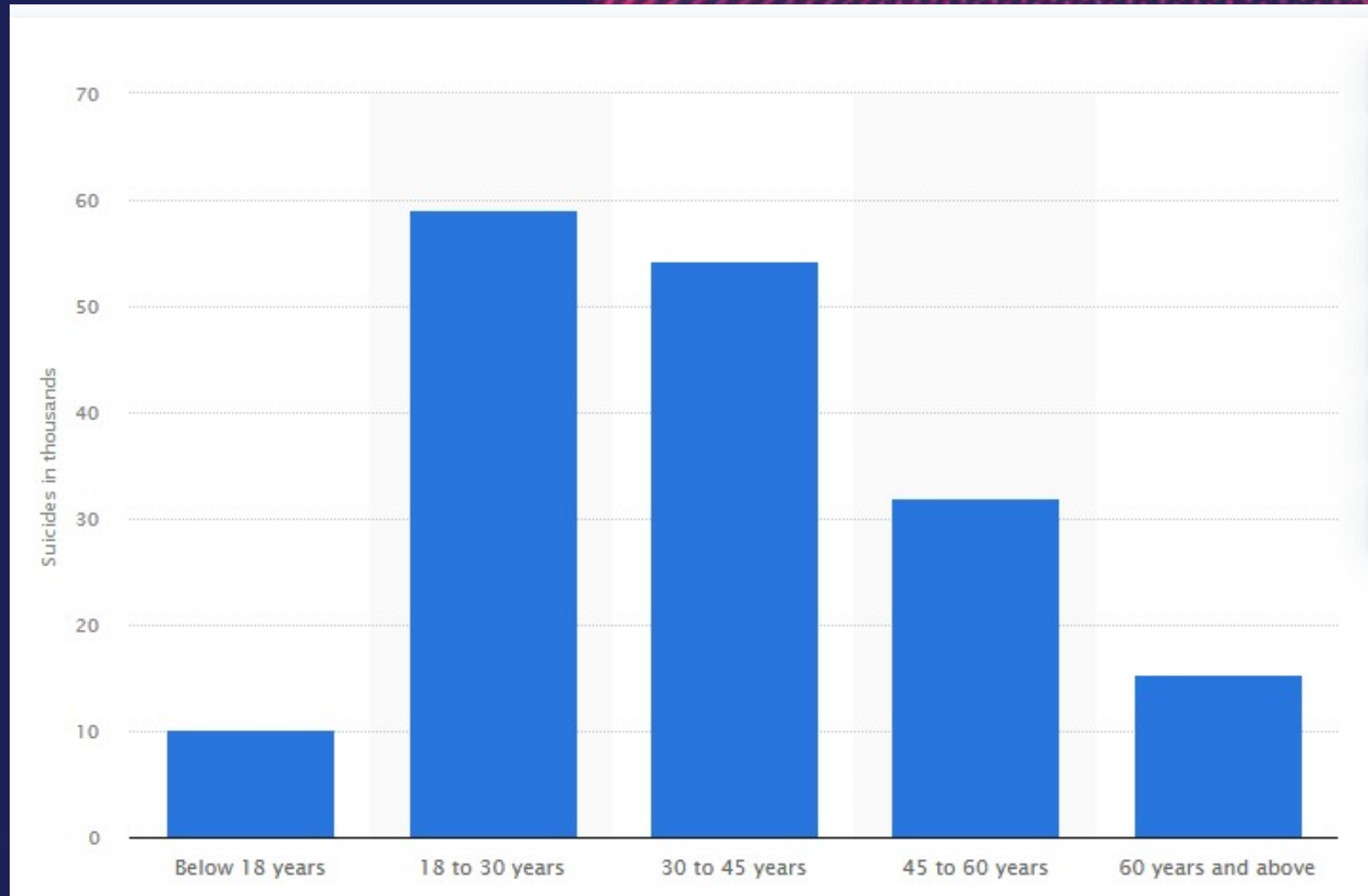
- There were 2,30,314 (95% UI 194 058–250 260) suicide deaths in India in 2016.
- India's contribution to global suicide deaths increased from 25·3% in 1990 to 36·6% in 2016 among women, and from 18·7% to 24·3% among men.

Number of suicides in India from 1971 to 2022

(in 1,000s)



PREVALENCE – NATIONWIDE BASED ON AGE



PREVALENCE – STATEWISE

States by Suicide Rate [\[edit \]](#)

Rank ↕	State ↕	Suicide Rate (Per 1 Lakh) 2020 ^[4] ↕	Suicide Rate (per 1 Lakh) 2015 ^[5] ↕
1	Sikkim	42.5	37.5
2	Chhattisgarh	26.4	27.7
3	Kerala	24.0	21.6
4	Tamil Nadu	22.2	22.8
5	Telangana	21.5	27.7
6	Tripura	20.9	19.6
7	Goa	19.9	15.4
8	Karnataka	18.4	17.4
9	Madhya Pradesh	17.4	13.3
10	Maharashtra	16.1	14.2
11	Haryana	13.7	13.0
12	West Bengal	13.4	15.7
13	Andhra Pradesh	13.4	12.1
14	Odisha	12.2	9.7
15	Gujarat	11.6	11.6
16	Himachal Pradesh	11.6	7.7
17	Arunachal Pradesh	10.5	10.4
18	Assam	9.3	10.0
19	Mizoram	8.9	11.7
20	Punjab	8.7	3.6
21	Uttarakhand	8.3	4.5
22	Rajasthan	7.2	4.8
23	Meghalaya	6.9	6.2
24	Jharkhand	5.6	2.5
25	Nagaland	2.2	0.9
26	Uttar Pradesh	2.1	2.0
27	Manipur	1.4	1.4
28	Bihar	0.7	0.5
UT1	Andaman and Nicobar Islands	45.0	28.9
UT2	Puducherry	26.3	43.2
UT3	Delhi	15.5	8.8
UT4	Dadra and Nagar Haveli and Daman and Diu	15.0	25.4
UT5	Chandigarh	10.7	6.9
UT6	Ladakh	4.0	3.0
UT7	Lakshwadeep	2.9	6.3
UT8	Jammu and Kashmir	2.2	3.0

SIGNS AND SYMPTOMS



- Talking about suicide — for example, making statements such as "I'm going to kill myself," "I wish I were dead" or "I wish I hadn't been born."
- Getting the means to take your own life, such as buying a gun or stockpiling pills.
- Withdrawing from social contact and wanting to be left alone.
- Having mood swings, such as being emotionally high one day and deeply discouraged the next
- Being preoccupied with death, dying or violence

SIGNS AND SYMPTOMS



- Feeling trapped or hopeless about a situation
- Increasing use of alcohol or drugs
- Changing normal routine, including eating or sleeping patterns
- Doing risky or self-destructive things, such as using drugs or driving recklessly
- Giving away belongings or getting affairs in order when there's no other logical explanation for doing this.

SIGNS AND SYMPTOMS

- Saying goodbye to people as if they won't be seen again.
- Developing personality changes or being severely anxious or agitated, particularly when experiencing some of the warning signs listed above.

Warning signs aren't always obvious, and they may vary from person to person.

Some people make their intentions clear, while others keep suicidal thoughts and feelings secret.



Who is at greater risk?

- The link between suicide and mental disorders (in particular, depression and alcohol use disorders) and a previous suicide attempt is well established in high-income countries
- Many suicides happen impulsively in moments of crisis with a breakdown in the ability to deal with life stresses, such as financial problems, relationship break-up or chronic pain and illness.
- In addition, experiencing conflict, disaster, violence, abuse, or loss and a sense of isolation are strongly associated with suicidal behaviour.
- Suicide rates are also high amongst vulnerable groups who experience discrimination, such as refugees and migrants; indigenous peoples; lesbian, gay, bisexual, transgender, intersex (LGBTI) persons; and prisoners.

DEPRESSION - PSYCHOTHERAPY

The aim of talking therapy is to relieve psychological distress through expressing your feelings and discussing your thoughts with a therapist.

It will help you better understand your own difficulties and guide you as you begin to identify and plan different ways and strategies for overcoming depression.

They may be delivered one-to-one or in a group, with your family, or with your partner, delivered face-to-face, online or over the phone.



DEPRESSION - PSYCHOTHERAPY

Research has found the following therapies to be effective in treating depression:

- Cognitive-Behavioural Therapy or CBT
- Counselling
- Psychodynamic Therapy
- Mindfulness Based Therapy
- Interpersonal Therapy
- Problem-Solving Therapy



It is important to understand that, whichever model of therapy is used, it normally takes some time before you begin to notice any changes.

DEPRESSION - MEDICATION

01

Selective serotonin
reuptake inhibitors
(SSRIs)

02

Serotonin and
Norepinephrine
reuptake inhibitors
(SNRIs)

03

Tricyclics and
Tricyclic-Related
drugs

04

Monoamine Oxidase
Inhibitors (MAOIs)

SUICIDE – PREVENTION & CONTROL

WHO GUIDELINES

01

Limit access to the means of suicide (e.g. pesticides, firearms, certain medications)

02

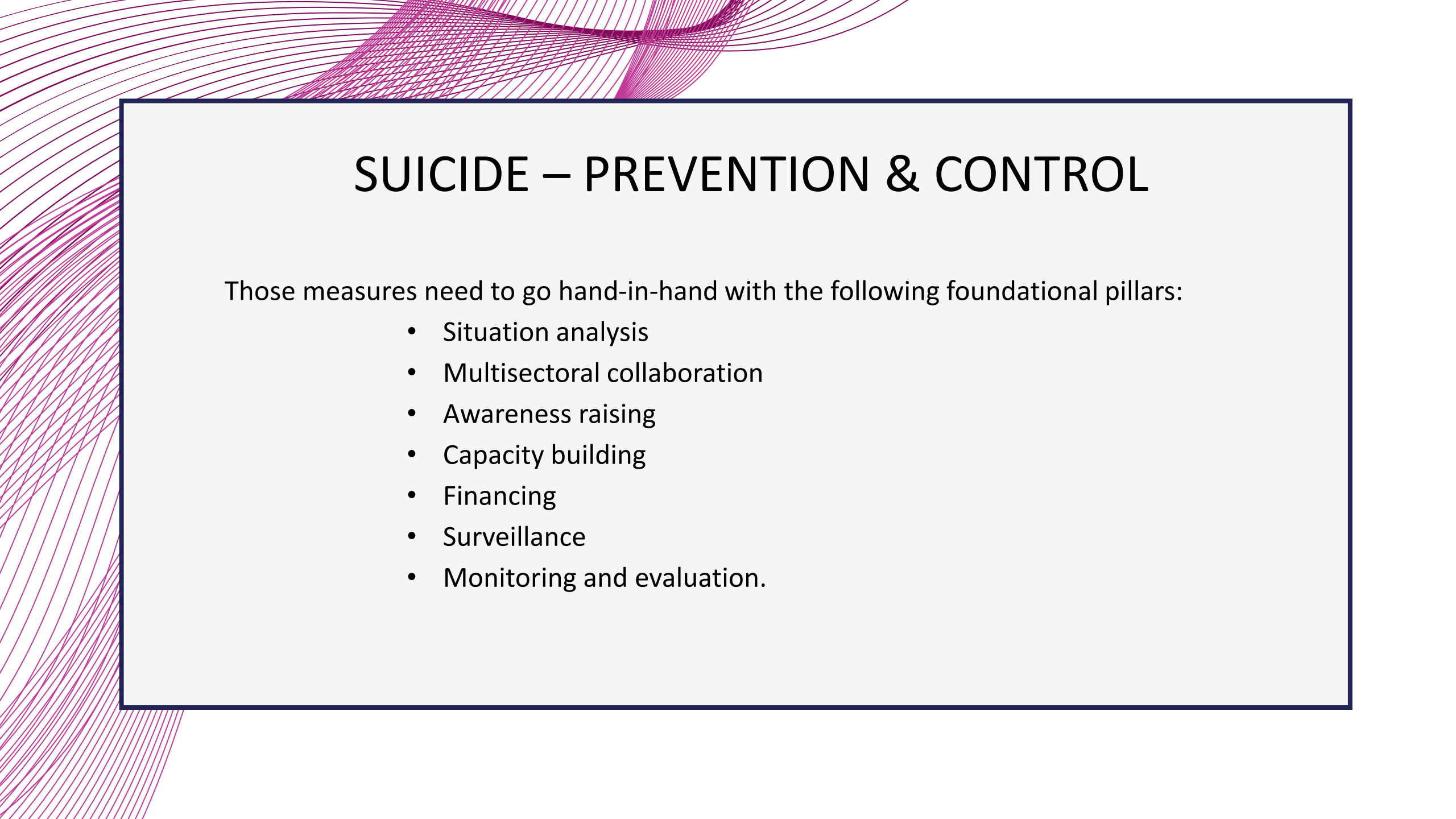
Interact with the media for responsible reporting of suicide

03

Foster socio-emotional life skills in adolescents

04

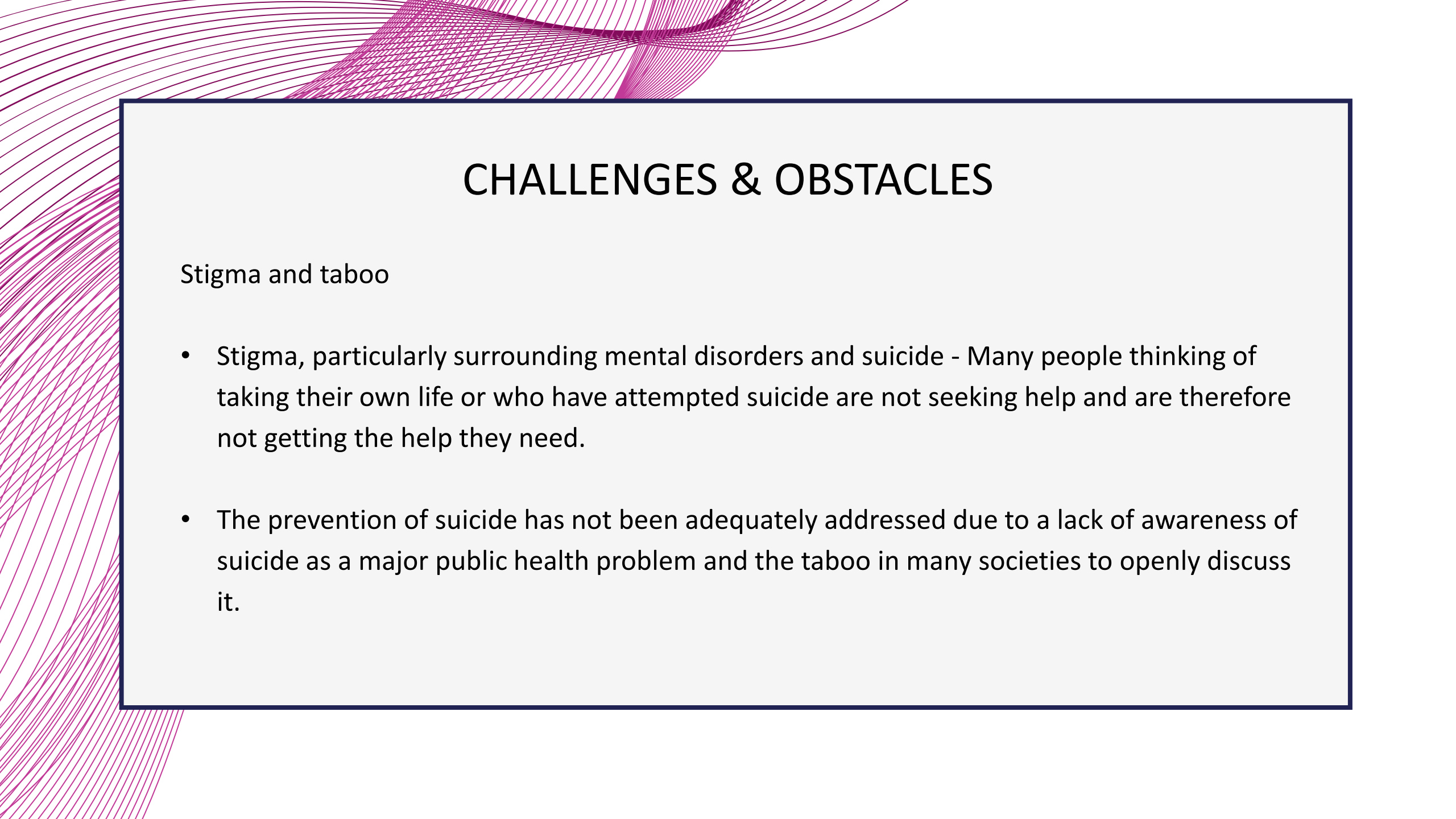
Early identify, assess, manage and follow up anyone who is affected by suicidal behaviors.



SUICIDE – PREVENTION & CONTROL

Those measures need to go hand-in-hand with the following foundational pillars:

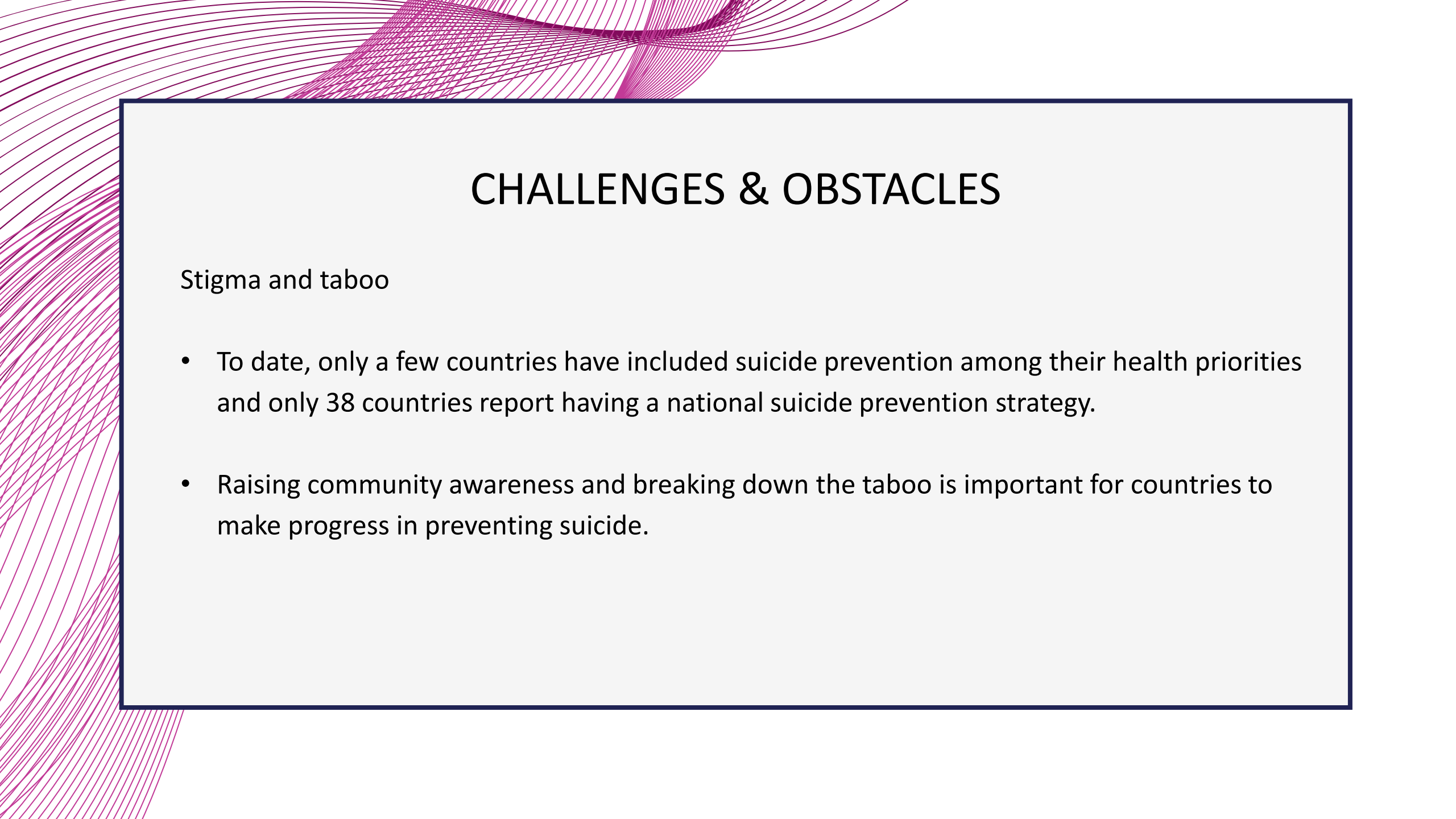
- Situation analysis
- Multisectoral collaboration
- Awareness raising
- Capacity building
- Financing
- Surveillance
- Monitoring and evaluation.



CHALLENGES & OBSTACLES

Stigma and taboo

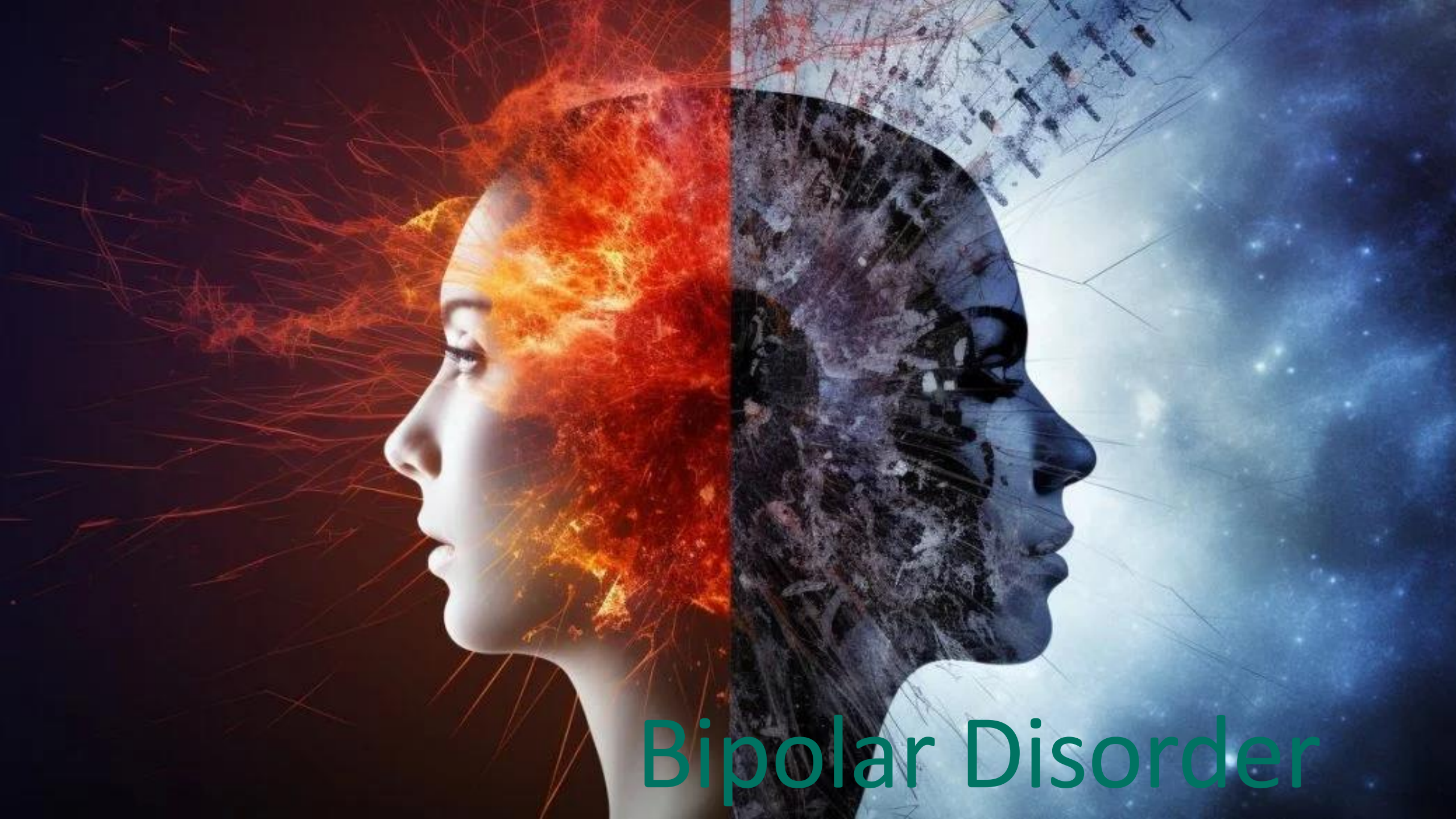
- Stigma, particularly surrounding mental disorders and suicide - Many people thinking of taking their own life or who have attempted suicide are not seeking help and are therefore not getting the help they need.
- The prevention of suicide has not been adequately addressed due to a lack of awareness of suicide as a major public health problem and the taboo in many societies to openly discuss it.



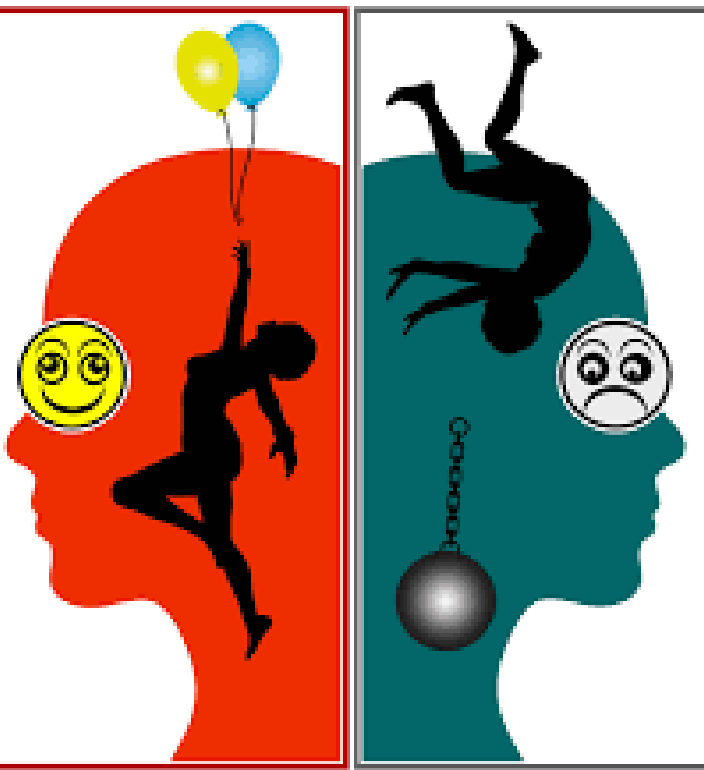
CHALLENGES & OBSTACLES

Stigma and taboo

- To date, only a few countries have included suicide prevention among their health priorities and only 38 countries report having a national suicide prevention strategy.
- Raising community awareness and breaking down the taboo is important for countries to make progress in preventing suicide.



Bipolar Disorder



It is as if my life were magically run by two electric currents: joyous positive and despairing negative - whichever is running at the moment dominates my life, floods it

Sylvia Plath (2000) *The Unabridged Journals of Sylvia Plath, 1950-1962* New York: Anchor Books

What is bipolar disorder?

Bipolar disorder, also known as manic-depressive illness, is a brain disorder that causes unusual shifts in mood, energy, activity levels, and the ability to carry out day-to-day tasks

Bipolar mood is characterized by recurrent episodes of mania and depression in the same patient at different times. • Earlier known as manic depressive psychosis (MDP)



ell

Types



Bipolar I - Characterized by episodes of severe mania and severe depression.



Bipolar II - Characterized by episodes of hypomania (not requiring hospitalization) and severe depression.



Depression phase



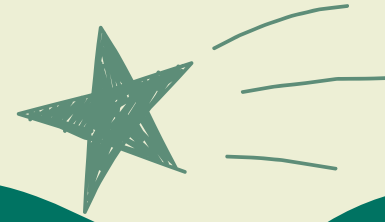
- Without treatment, a person with bipolar disorder may have intense episodes of depression.
- Symptoms include sadness, anxiety, loss of energy, hopelessness, and trouble concentrating.
- They may lose interest in activities that they used to enjoy.
- It's also common to gain or lose weight, sleep too much or too little, and even think about suicide

Symptoms of Depression



- Prolonged sadness or unexplained crying spells
- Loss of appetite and changes in sleep patterns with sleeping far too much
- Irritability, anger, worry, agitation, anxiety
- Pessimism, indifference, defeat
- Loss of energy
- Feelings of guilt, worthlessness
- Having problems focusing, remembering, and making decisions
- Unable to enjoy things anymore, social withdrawal and isolation
- Unexplained aches and pains
- Suicidal thoughts or attempting suicide.

Manic phase



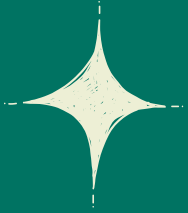
- During this phase, people feel super-charged and think they can do anything.
- Their self-esteem soars out of control and it's hard for them to sit still.
- They talk more, are easily distracted, their thoughts race, and they don't sleep enough.
- It often leads to reckless behavior, such as cheating, fast driving, and substance abuse.
- Three or more of these symptoms nearly every day for a week accompanied by feelings of intense excitement may signal a manic episode.

Symptoms of Mania



- Excessive energy, racing thoughts and rapid talking
- Denial that anything is wrong
- Easily irritated or distracted
- Decreased need for sleep – possibly days with little or no sleep without feeling tired
- Unrealistic beliefs in one's ability and powers
- Unusually poor judgment
- Abuse of drugs and alcohol
- Provocative, intrusive, or aggressive behavior

Diagnostic Guidelines acc to DSM - 5



For a diagnosis of bipolar I disorder, it is necessary to meet the following criteria for a manic episode.

The manic episode may have been preceded by and may be followed by hypomanic or major depressive episodes.



Diagnostic Guidelines acc to DSM - 5

Manic Episode

A. A distinct period of abnormally and persistently elevated/irritable mood and increased activity/energy, lasting at least 1 week and present most of the day, nearly every day

B. During the period of mood disturbance and increased energy or activity, three (or more) of the following symptoms

1. Inflated self-esteem or grandiosity.
2. Decreased need for sleep (e.g., feels rested after only 3 hours of sleep).

Diagnostic Guidelines acc to DSM - 5

3. More talkative than usual or pressure to keep talking.
4. Flight of ideas or subjective experience that thoughts are racing.
5. Distractibility as reported or observed.
6. Increase in goal-directed activity (either socially, at work or school, or sexually) or psychomotor agitation (i.e., purposeless non-goal-directed activity).
7. Excessive involvement in activities that have a high potential for painful consequences

Diagnostic Guidelines acc to DSM - 5

C. The mood disturbance is sufficiently severe to cause marked impairment in social or occupational functioning or to necessitate hospitalization

D. The episode is not attributable to the physiological effects of a substance or another medical condition.

Diagnostic Guidelines acc to DSM - 5

Major Depressive Episode

A. Five (or more) of the following symptoms have been present during the same 2 week period and represent a change from previous functioning; at least one of the symptoms is either (1) depressed mood or (2) loss of interest or pleasure.

1. Depressed mood most of the day, nearly every day, as indicated by either subjective report or observation made by others

Diagnostic Guidelines acc to DSM - 5

2. Markedly diminished interest or pleasure in all, or almost all, activities most of the day, nearly every day
3. Significant weight loss when not dieting or weight gain or decrease or increase in appetite nearly every day.
4. Insomnia or hypersomnia nearly every day.
5. Psychomotor agitation or retardation nearly every day
6. Fatigue or loss of energy nearly every day.
7. Feelings of worthlessness or excessive or inappropriate guilt nearly every day

Diagnostic Guidelines acc to DSM - 5

- 8. Diminished ability to think or concentrate, or indecisiveness, nearly every day
- 9. Recurrent thoughts of death, recurrent suicidal ideation with/without a specific plan, or a suicide attempt
- B. The symptoms cause clinically significant impairment in social, occupational functioning.
- C. The episode is not attributable to the physiological effects of a substance or another medical condition.

Therapy



1. Cognitive Behavioral Therapy (CBT) - Helps people with bipolar disorder learn to change harmful or negative thought patterns and behaviors.
 - Its a short-term psychotherapeutic individual intervention designed for treating depression.
 - It is based on cognitive restructuring and is aimed at decreasing depressive symptoms and improving self-esteem.
 - It includes self-monitoring and self-regulation, by means of managing and dealing with automatic, dysfunctional thoughts, and usually includes behavioral techniques for decreasing environmental stress & promote social adaptation .
 - There is substantial evidence for the effectiveness of CBT for depression.
 - The application of this theory for bpd derives from this research.
 - There is evidence that improvements in mood and social functioning have been made with individual CBT.

Therapy



2. Family-focused interventions:

It addresses enhancing communication and coping skills, as well as the role of expressed emotion amongst families.

Studies seem to indicate some decreases in relapse rates.



Therapy

3.

Interpersonal and Social Rhythm Therapy -

It helps people with bipolar disorder improve their relationships with others and manage their daily routines.



Therapy

4. Schema-focused therapy -

- Schemas are core beliefs/pervasive themes regarding oneself and others.
- They are self-perpetuating, with an individual tending to distort information to maintain its validity.
- The modified schema-focused cognitive therapy incorporates schemas associated with adaptability to illness and adaptability styles.



Therapy

5. Psychoeducation -

- Usually done in a group, teaches people about the illness and its treatment.
- Can help to recognize signs of an impending mood swing so treatment can be sought early, before a full-blown episode occurs.
- It may also be helpful for family members and caregivers
- It tend to include 2-9 sessions that are mainly informative about the disease and its pharmacological treatment.
- Its goal is to define bp illness as a biological disturbance and to focus treatment on pharmacological measures.



Medications



There is no one medication for bipolar disorder. Depending on the person, various combinations of these or similar drugs might be prescribed.

- Mood stabilizers:
- Antidepressants:

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). <https://doi.org/10.1176/appi.books.9780890425787>

